

Quality Management System Overview

Loblolly Ridge Therapeutics | Quality Assurance | 2026-02-02

An overview of the Loblolly Ridge quality management system: what it covers, where GxP rules apply to our work, how controlled documents are organized, the core quality processes, who is responsible and how management reviews the system. Revision 5 reflects the move to the eQMS in November 2025. The detailed requirements are in the SOPs listed in the eQMS.

Scope

The quality management system covers GCP, GLP oversight, GMP oversight and pharmacovigilance activities performed or sponsored by Loblolly Ridge, and computerized-system validation for GxP systems. Discovery research is outside GxP scope.

Where GxP applies to our work

Clinical studies are run under GCP. We are the sponsor; our CRO and other service providers perform transferred activities under our oversight. Nonclinical safety studies that support regulatory submissions are done under GLP at qualified external testing facilities, each with a quality assurance unit independent of the study staff; we oversee them but do not run a GLP facility. Manufacturing and testing of clinical material are done under GMP at CDMOs and contract laboratories; our QA releases material for clinical use.

Pharmacovigilance is GxP work shared with our safety database provider. GxP computerized systems, including the eQMS, are validated. Research at the Woodlands laboratory is non-GxP, but research records are kept attributable, contemporaneous and reviewed as good practice.

Outsourced activities

Most GxP work is done by partners. ICH Q10 makes the company responsible for controlling outsourced activities, including assessing suppliers before use, written agreements and monitoring performance. Partners are risk-tiered: tier 1 partners, the GxP-critical ones, are audited before first use and every 24 months; tier 2 every 36 months or by questionnaire; tier 3 have no routine audit. The 2026 audit program covers tier 1 partners due for routine audit, for-cause audits and an internal audit of TMF oversight.

Document hierarchy

Controlled documents run from policies to SOPs, work instructions, and forms and templates. SOPs and policies are reviewed every 24 months; forms and research work instructions every 36 months. Since November 17, 2025 all controlled documents, training records, deviations and CAPA are managed in the eQMS, which replaced a controlled file share and paper training records.

Core quality processes

Document control and periodic review.

Training: role-based curricula assigned and recorded in the eQMS.

Deviations, audit findings and supplier notifications, classified minor, major or critical. A CAPA is required for major and critical events and for repeated minor ones; effectiveness checks follow CAPA closure.

Change control for GxP processes and systems.

Partner qualification, audits and oversight.

Data integrity: FDA defines data integrity as the completeness, consistency and accuracy of data; our GxP systems keep secure, time-stamped audit trails.

Release of clinical material.

Roles

Head of Quality: owns the quality management system and reports on it to the CEO.

QA GCP and pharmacovigilance lead: clinical and safety oversight audits.

QA GMP lead: CDMO oversight and material release.

QA GLP oversight: deputy role to be assigned; covered by the Head of Quality in the meantime.

Function heads: follow the procedures that apply to their teams and own their controlled documents.

Management review

Quality Assurance holds an annual management review each February, chaired by the CEO with the executive team, covering audits, quality events and CAPA, training, document control, partner quality and resources. The review sets the quality objectives for the year and records decisions and resource needs.